

connected with the Self are more willing to unblend in the future, both in session and at home.

As ranting managers calm down and firefighters let up on high-risk practices, the mind feels more spacious, and parts trust the Self more. At this phase of treatment, we see clients begin to make different choices on occasion. However, these periods of unblended psychic space also give exiles the opportunity to push for attention, which can cause protectors to double back and panic. To head off the urgency of exiles, we continue to include them in brief *one-way* connections to the Self, reminding them they are not alone and that we are coming with help. We promise that addressing the concerns of tough protectors will help to create more space and safety for them. We ask them not to overwhelm the system, which is an intervention Schwartz discovered early on in his work with exiled parts (Schwartz & Sweezy, 2020).

As the client invites protectors to be direct about their worries, they gradually pivot from acting out to saying no, saying yes, and making requests. If protectors believe therapy is moving too fast, we can slow down. These conversations around speed and need are novel for parts who are used to being disregarded and disempowered. They are often crucial turning points in the client's inner journey.

GETTING TO KNOW FIREFIGHTERS

By the time clients with compulsivity problems walk into our office, they will have tried countless times—sometimes with multiple strategies—to control their drinking, eating, sexual obsessions, or online gambling. Some of their parts will be angry about being shamed, controlled, or judged during bad experiences with previous treatments (Hari, 2015). Others will be fearful of failing again or even ready to die before they tolerate one more humiliation. When an IFS therapist is able to notice these negative assumptions and guide these parts to step back, therapy will feel safe for the client and their parts will engage. A firefighter part might start dropping hints, like complaining that other people think their behavior is getting worse, or a manager might spill the beans more directly. Either way, this information gives us an opportunity to move on from the addiction polarity and review the client's earliest experiments with firefighter behaviors.

We recommend coupling this review with lots of appreciation for a couple of reasons. First, firefighters are remarkably perceptive. They see that the harsh, self-loathing tirades of managers who are trapped in a grim devotion to obligation lead to more loneliness, shamefulness, and despair. Their concerns are legitimate and reality-based. The client's internal environment really is too harsh. In turn, firefighters aim to balance *shoulds*—too much caretaking, too much responsibility, too much shaming, too few resources—with some *wants* that distract from emotional pain and loneliness. However chaotic and self-damaging a firefighter's behavior, it provides real relief. When the client is able to be curious, they learn that firefighters ignore long-term negative consequences because they are upholding one side of a potentially lifesaving (if precarious) bargain.

Second, we make a point of appreciating firefighters because clients have a wide variety of priorities when it comes to change. For example, they may want to work on their bulimia but not feel ready or interested in changing their drinking patterns. As with harm reduction approaches, we subscribe to the idea that recovery is any positive change a client makes (Szalavitz, 2016; Tatarsky, 2007). Our first priority is not controlling compulsivity but getting in relationship and building healthy inner connections between firefighter parts and the client's Self. Additionally, we invite the client to identify how their addictive parts are serving them at this time, and we solicit information about other firefighter activities that feel beneficial, like hanging out with friends, walking the dog, playing an instrument for fun, racing motorcycles, submerging into a novel, bingeing on a TV series, playing video games, or going to a movie.

Since we aim for the client to see their risk-taking behavior in a new light, we recommend pursuing the firefighter review gently. We hope they will learn to understand (rather than be critical of) their

addictive behaviors and they will appreciate (rather than fear) the depth of their need for relief. Toward this end, it is important that we head off potential backlash from critical, shaming managers before we do a firefighter review by asking about their concerns. Managers rightfully fear getting friendlier with firefighters. They don't trust the Self yet and assume that firefighters will interpret any rapport as permission and endorsement of what they do. We gain manager trust by responding to their legitimate fears and concerns. To do so, use the following steps.

ASK, VALIDATE, ASK

First, ask inside if any part objects: "Does any part feel worried or object to you talking about your early experiences with me?"

- If a firefighter part fears that the therapist (or another part internally) will judge its behaviors, ask about its intentions ("What would have happened without you?") and validate its sense of necessity.
- If a manager part fears that the therapist (or another part internally) will not judge firefighter behaviors, validate this concern but stand up for the firefighter: "This firefighter part is intense, but not crazy. It tried to help that kid escape a really painful time when no one else was helping."

Next, ask about manager fears: "What would happen if you didn't say critical things right now?" It is important to note their validity of these worries—as managers have seen the past behaviors of other parts. Typically, critical managers have one or all of these concerns:

- The exile will blend with and overwhelm the system.
 - Validate the client's experience of this in the past, and offer to help exiles so they don't need to overwhelm again.
- The firefighter will take over.
 - Validate that firefighters have run the show many times with disastrous consequences, and promise that firefighters are more complex than they seem so they, too, can be helped.
- The manager thinks it is the job it does, so it will disappear if the job ends.
 - Validate the manager's hard work, and promise that the client's Self will have no interest in getting rid of any part. You are just offering the part a chance to worry and work less.
- The manager fears that someone else will get mad and reject the client.

- Validate that the client has been shamed, rejected, or disowned many times before, and promise to support the client's new process not to judge or control anyone.

After reassuring managers and securing their permission to proceed, explore the client's earliest history of substance use or other high-risk behavior. Follow up on hints or direct confessions with nonjudgmental, open-ended questions. Start by inviting them to think of when they first took a major risk, got in big trouble, or felt like they were acting out. Ask out how old they were at the time and inquire about their circumstances.

For example, if the client smoked a cigarette, shoplifted a sandwich, lied to get out of trouble, played cards for money, looked at porn on their dad's computer, started dieting, had sex for the first time, or in some way rebelled more openly, what was going on in their life? How old were they? Depending on a client's environment and history, this rebellion could have happened when they were a teenager, or even in grade school. As they revisit the past, help them connect with their young protectors and listen to their intentions. You will learn the level of risk they faced at home and whether any adult was available to guide or protect them. You may hear about parents, preoccupied with work or with other relationships, who were stuck in their own addictive cycles or who were emotionally, physically, or sexually abusive. You may hear about chronic chaos, lack of supervision, food insecurity, or loneliness.

For some clients, this pain and loss may be more subtle. Their environment may have been quite secure and well-resourced, yet they still felt unloved, were chronically anxious about their performance or appearance, or were always invalidated and thus felt unsafe being assertive. Each client has a story. Whatever their childhood circumstance, they gain clarity by looking at the positive intentions of their parts—whether it was an eight-year-old firefighter part who got entry to the “big kid” crowd by smoking cigarettes; a ten-year-old part who was left home alone, comfort eating junk food in front of the TV; a thirteen-year-old part who soothed their rage at school bullies by cutting their arms and inner thighs; or a fifteen-year-old part who began to boycott food after being humiliated at dinner.

As author Maia Szalavitz (2016) wrote in her book *Unbroken Brain*, “Addiction doesn't just appear; it unfolds” (p. 38). We understand how addictive processes develop throughout a client's life, but when the client listens to their firefighter parts, the big picture comes into focus more clearly before their eyes. They see their childhood vulnerability and understand how it led to the clever strategies of their young protectors. When the client can see how lonesome, neglected, or scared their young parts were, the client has a first shot at self-compassion: *I wasn't bad. I was desperate!*

FIREFIGHTER HISTORY REVIEW QUESTIONS

- Do you remember the first time you tried something risky in your neighborhood?
In your home?
- Do you remember ever wanting to get away with something and not get caught?
- How was school? Did you participate in outside activities?

- Who was home when you got home from school?
- Were you ever home alone? If so, where were your caregivers or siblings?
- What did you do when you got home from school?
- Were you able to relax in your home? If so, how did you relax?
- Could you relax in your neighborhood? Did you feel safe outside of your apartment building or home?
- Did you have fun when you were young? If so, what was fun for you?
- Were you ever nervous or scared in your family? At school? Was anyone there to help you with those fears?
- How did your immediate family function? Did your caregivers ever use drugs, drink alcohol, have eating issues, become inappropriate sexually, or get violent?
- Did you feel comfortable being yourself with family?
- Did you feel the need to hide any parts of yourself or to hide any of your favorite activities at home or in school?
- What was your extended family like?
- Did you have responsibilities at home? If so, at what age?
- Did you have enough food to eat? What kind of food was it?
- How old were you when you started to diet, gamble, use substances, or engage in other risk-taking behaviors? What was happening at the time?
- Are you hearing anything critical about your young self who _____ (e.g., got high, smoked cigarettes, looked at porn)? If so, what is the critical part afraid would happen if it didn't say these things right now?

Expect that many clients will be disconnected from their needs and motives when you conduct a firefighter review. This might be the first time they notice their childhood vulnerability. That's because, as we've learned, many clients who are engaging in addictive practices come from chaotic or disengaged family systems and learned to disconnect from their needs at a young age, a state that grows over time. As a result, they are not versed in self-care and self-attunement, and they don't easily associate early deprivation and wounding with their addictive process. For example, they don't realize they got high yesterday because they felt overwhelmingly isolated. They don't realize they

restricted their food intake over the weekend because they were overtaken by a deep feeling of shamefulness and sense of alienation. Rather, they're most likely to say they had a rotten week and simply felt like giving in to the urge.

Although we might conclude that these clients are avoiding reality and accountability, they are not aware that they are making choices to manage their discomfort and pain. Rather, they report not thinking at all. They're genuinely mystified by their compulsivity and impulsivity. The behavior happens to them—again and again—and they don't see the larger context for their actions.

So how can a therapist intervene effectively when a client has so little self-awareness and sense of self-agency? We start by asking questions that invite clients to notice their patterns and protector polarities. We continue by painting the big picture, pointing out that their protective parts are divided into two opposing teams, and helping them notice how vulnerable they were when they turned to distracting and self-medicating.

To see how this looks in practice, consider the example of Sonia, a thirty-two-year-old, African American, single, heterosexual, cisgender woman, who came to therapy because, as a newly minted nurse, she had an urge to steal drugs on the job.

WORKING WITH YOUNG PROTECTORS



Therapist: You mentioned things were not great at home when you were young. Do you remember how old you were when you first tried alcohol?

Sonia: Eleven. We took beers from the fridge in the neighbor's garage.

Therapist: Is that also when you started smoking cigarettes?

Sonia: I was ten when I started smoking. Me and this kid in my neighborhood would go to the park and smoke his mother's cigarettes. Sometimes we played around, too. We didn't really know what we were doing. But I guess it was kind of nice just being together.

Therapist: It was good to have company. What else was going on when you were ten and eleven years old?

The therapist validates and inquires about her environment.

Sonia: My dad left and my mom was depressed. She stayed in bed. She was drinking, though she thought we didn't know. She hid the booze in a boot in her closet.

Therapist: And what were you and your younger siblings doing?

Sonia: Mike kinda moved in with the family next door. That left me with little Pete and Mom. I begged for money from relatives in the neighborhood and did the food shopping and took care of little Pete. Sometimes the lady across the street left containers of leftover food on our doormat. And when little Pete got burned, I took him to the hospital.

Therapist: How?

Sonia: I picked him up and ran. Then the government people came. That was when we went to foster care.

Therapist: Sonia, when you talk about this, what do you hear inside?

Sonia: *You let little Pete get hurt. You let those people take him away.*

This is Sonia's critical manager team.

Therapist: Did you and your brothers go to the same foster placement?

Sonia: We did. But Mike snuck out of the foster home and went back to his friend's house. They let him stay there. Then Aunt Caroline took little Pete, and I eventually went back to Mom.

Therapist: When did you go back?

Sonia: After about a year, I think. She got sober for a while.

Therapist: So you have a part who says little Pete getting burned and the government people coming to take you all away was your fault.

The therapist introduces parts language.

Sonia: It was. But then I got a boyfriend, Marty, when I got home. He was sixteen. I was fourteen. We had fun while it lasted. He got vodka from his older brother who worked in a liquor store. *(She shrugs.)* What can I say? I was a bad kid. I dropped out of school too many times to count. It was a miracle I finished.

Therapist: Your critical part sees that kid on her own and says she was bad. Let the part know that she wasn't bad. I'm hearing she was a kid in the worst kind of circumstance. She had all that responsibility and no grown-ups to help. She was too young to keep her little brother safe. Goodness! She needed someone to take care of her. Then she got put in a stranger's house, she was all alone, and she blamed herself. She needed some comfort and relief. Cigarettes were a start, but Marty and alcohol were even more effective.

Sonia: *(She tears up.)* I never saw little Pete again. I begged my aunt to take me too, but she said she couldn't.

Therapist: And what did the critic inside you say when that didn't happen?

Sonia: *No one will ever love you. You're a selfish, nasty girl.*

Therapist: Does the critic still say stuff like that? *(Sonia nods.)* What is it concerned would happen if it stopped?

Sonia: *(She hangs her head.)* I'd be too much.

Therapist: And?

Sonia: And no one would ever love me.

Therapist: Do you agree, Sonia? *(She nods, looking at the floor.)* Okay, so let the critical part know that, yes, there was a young girl who had all that responsibility and no grown-up help. Her mother was ill, and she tried to keep her little brother safe, but she was too young. Who was taking care of her? She was vulnerable too. She was alone and scared, and she had a critic beating her up inside. *(pausing)* Can you see her in that foster home?

Sonia: I see her... She's sad.

This is the exile.

Therapist: Is she aware of you? (*Sonia nods.*) Let her know you're here now. Would she like your help?

Sonia: Yes.

Therapist: Tell her we'll come back to her soon. First, we have to check in with the parts who drank and smoked and so forth to get their permission. Can you see them too?

Sonia: I see the fourteen-year-old. She needed Marty and the booze.

This is a firefighter part.

Therapist: Do you get that she needed Marty and the booze? (*Sonia nods.*) Would she like your help too?

Sonia: Yes.

Therapist: Okay, great. We'll come back to her.



Since Sonia was convinced that she was bad as a child (and was still bad now), it was clear that her judging, critical managers were still present. At the same time, she felt unlovable, so her exiles were also in her consciousness. This is a formula for firefighter intervention that can lead to a recurrence of compulsive behavior. While many clients like Sonia can readily, even dispassionately, recount a devastating family history, we should not mistake that dispassion for calm and self-compassion. Their protectors have no idea how to meet the needs of traumatized, abandoned parts. In IFS, we assert that it's never too late to heal attachment wounds and bring exiles to a safer place. We make sure firefighters have a chance to explain their good intentions, and we validate the legitimate cravings of exiled parts for comfort, warmth, adult protection, and safety.

ASKING ABOUT OUTSIDE INFLUENCES

Many clients trying to control their addictive processes are subject to external pressure from loved ones. Their behavior is chaotic, they habitually disappoint others, they lack follow-through, and they have legal and medical problems. All this recruits the manager parts of friends and family, and also of the health care professionals who cycle through their life trying to help. All of these well-intentioned people (including us!) explain, shame, threaten, manipulate, beg, and nail down contracts in which the client agrees to minimize or stop the addictive behavior. However, this external pressure won't end the chaos, stop the cycle of self-destruction, or effect a cure. In contrast, it will motivate firefighters to resist intervention and act out more stridently.

Of course,